Fentanyl

****This monograph is under review****

Neonatal

FORM

Ampoule containing 100microgram in 2ml
Syringe containing 25microgram in 2.5ml

INDICATION

a) Premedication prior to intubation

b) Analgesia/Sedation/Respiratory depressant for infants receiving assisted ventilation

DOSE RANGE

a) Premedication prior to intubation

AGE	DOSE	FREQUENCY	ROUTE
Birth-6 months	5microgram/kg	Single Dose	IV over 60 seconds with 60 seconds flush

b) Analgesia/Sedation/Respiratory depressant for infants receiving assisted ventilation

AGE	DOSE	FREQUENCY	ROUTE
Birth to 6 months	1-5microgram/kg	Initial dose	IV
	1-3microgram/kg	Supplemental doses as necessary	IV

RECONSTITUTION

Already in solution

DILUTION

Using ready prepared syringe (premedication only)

A ready prepared syringe of fentanyl 10microgram/ml is available as part of the neonatal intubation drug kit (CD fridge)

Dosage guide for intubation kit syringe:

- 10microgram in 1ml
- 5microgram in 0.5ml
- The premedication dose is 0.5ml/kg of above solution
- No further dilution is required

Using ampoules

Fentanyl injection 50microgram/ml	0.5ml
Sodium Chloride 0.9%, 0.45% or Glucose 5%	up to 2.5ml total

This gives a 10microgram/ml solution

METHOD OF ADMINISTRATION

Give by slow intravenous injection over 60 seconds followed by a 0.5-1ml flush over 60 seconds

COMPATIBILITY

Solution compatibility	Glucose 5%, Sodium Chloride 0.9%, Sodium Chloride 0.45%
Solution incompatibility	No information
IV Line compatibility	Caffeine citrate
IV Line incompatibility	No information

THIS LIST IS NOT EXHAUSTIVE PLEASE CONTACT PHARMACY FOR FURTHER INFORMATION ON COMPATIBILITY WITH ANY MEDICINES NOT INCLUDED

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS
- See Summary of Product Characteristics and most recent edition of BNF for Children

FURTHER INFORMATION

Use with caution if there is known renal impairment
Use 50% of standard dose if there is severe renal impairment
(i.e. Oliguria <0.5ml/kg/hour, or Creatinine >120µmol/l or increasing >30µmol/l in 24hrs).
Follow dose with a sodium chloride 0.9% flush over - 60 seconds

Chest wall rigidity may occur, but is less likely if administered as a slow bolus and resolves with paralysis. *If the baby develops chest wall rigidity, this can be reversed with suxamethonium and this should be bolused immediately.*

STORAGE

Store syringes in a locked fridge.
Store ampoules in the ward controlled drug cupboard.

PH

3.5-7.5

LICENSED STATUS

Not licensed for children <2 years
Prefilled syringes – unlicensed

APPLICABLE POLICIES

West of Scotland Neonatal Guidelines

Consult local policy if applicable

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Administer reconstituted solutions immediately.
All vials, ampoules and infusion bags are for single use only unless otherwise stated.

Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Network. Information is correct at the time of publication and as per local practice agreement.
For further advice please contact your clinical pharmacist or pharmacy department